

Excellence Health System

Document Type: ED Provider Note

Patient Name: John Doe
MRN: 94817263

DOB: 1952-11-04
Date of Service: 2026-08-08

CHIEF COMPLAINT: Shortness of breath, worsening over 3 days.

HISTORY OF PRESENT ILLNESS:

71-year-old male with a history of HFrEF (baseline EF 25%) presenting to the ED with worsening dyspnea at rest, orthopnea, and lower extremity edema.

Patient reports skipping his Lasix doses for the past week.

PHYSICAL EXAM:

Vitals: BP 168/94, HR 110, RR 26, O2 Sat 88% on room air.

Resp: Bilateral crackles heard halfway up lung bases.

CV: Tachycardic, regular rhythm, JVD present.

Extremities: 3+ pitting edema bilaterally to the knees.

LABORATORY & IMAGING:

BNP: 2,400 pg/mL (elevated)

Creatinine: 1.8 mg/dL

Troponin: 0.04 ng/mL

CXR: Bilateral pleural effusions and pulmonary vascular congestion.

ASSESSMENT & PLAN:

Acute on chronic systolic heart failure exacerbation. Severe fluid overload.

Hypoxic requiring supplemental oxygen.

- Start IV Lasix 80mg.
- Admit to inpatient telemetry.
- Strict I&O, fluid restriction.